

Low-Dose Aspirin & Preeclampsia Prevention

UNDERSTANDING YOUR RISK · EVIDENCE-BASED PREVENTION · WHAT TO WATCH FOR

ACOG
Endorsed
Guideline

WHAT IS PREECLAMPSIA?

- ▶ High blood pressure that develops **after the 20th week** of pregnancy — or after giving birth
- ▶ Involves signs of **organ stress** (kidneys, liver) alongside elevated BP
- ▶ Most women with preeclampsia have **healthy babies** with proper monitoring
- ▶ Left untreated, it can cause **premature birth, organ damage, and death**

2–8%
of pregnancies
worldwide

15%
of premature births
in the US

20wk
earliest it
typically develops

HOW LOW-DOSE ASPIRIN HELPS

- ▶ Baby aspirin = **81 mg** — this is the dose your doctor prescribes
- ▶ Reduces risk by improving blood flow to the placenta and reducing inflammation
- ▶ **ACOG-endorsed**: safe in pregnancy, low risk of serious complications
- ▶ Recommended starting **after 12 weeks** for high-risk patients
- ▶ Take **exactly as directed** — do not take more, do not skip doses
- ▶ Available **over the counter** or by prescription

- 1 **When:** Begin after 12 weeks — your provider will tell you the exact start date
- 2 **How:** Take 1 tablet (81 mg) daily — evening preferred to reduce GI upset
- 3 **Until:** Continue every day through delivery unless told otherwise

WHO IS AT RISK? — RISK FACTOR GUIDE

● HIGH RISK — ASPIRIN STRONGLY RECOMMENDED

- Previous preeclampsia (especially early-onset)
- Multiple gestation — twins, triplets, or more
- Chronic hypertension (high blood pressure before pregnancy)
- Diabetes (Type 1, Type 2, or gestational in prior pregnancy)
- Chronic kidney disease
- Autoimmune disease: lupus, antiphospholipid syndrome

● MODERATE RISK — DISCUSS WITH YOUR PROVIDER

- First pregnancy or more than 10 years since last baby
- Obesity (BMI ≥ 30)
- Family history of preeclampsia
- Prior pregnancy with low birthweight baby
- IVF / assisted reproductive technology conception
- Age > 35 · African-American race · Low socioeconomic status

* Having 1 high-risk factor or multiple moderate-risk factors may qualify you for low-dose aspirin therapy. Your provider will decide what's right for you.

CALL YOUR PROVIDER IMMEDIATELY IF YOU HAVE ANY OF THESE SIGNS

 **Blurred Vision**
or seeing spots / flashing lights

 **Severe Headache**
that won't go away with rest

 **Swelling in Hands or Face**
especially sudden or severe

 **Severe Belly Pain**
upper right side or stomach area

DR. KLEIN'S RECOMMENDATION



Bayer Low-Dose Aspirin 81mg — available over the counter at any pharmacy without a prescription. Take **one tablet daily** starting after 12 weeks and continue through delivery. Do not take extra aspirin or NSAIDs (like ibuprofen) without checking with us first.

81mg Only

Available OTC

Take Daily

Continue Until Delivery

ACOG Guideline

KEEP ALL PRENATAL VISITS

- ▶ Attend **every scheduled appointment** — even when you feel completely fine
- ▶ Blood pressure is **checked at every visit** to catch changes early
- ▶ Preeclampsia can develop **without obvious symptoms** at first
- ▶ Never skip a visit — early detection saves lives

 **BP Check
Every Visit**

 **Urine & Labs
As Ordered**

 **Growth Scans
If Indicated**

 **NST / BPP
If Indicated**

KEY REMINDERS AT A GLANCE

- ▶ Low-dose aspirin is **safe** when used as directed during pregnancy
- ▶ Do **not** take regular aspirin (325mg) — only the 81mg dose
- ▶ Do **not** stop taking aspirin without talking to your provider first
- ▶ Aspirin works best when started **before 16 weeks** gestation
- ▶ Monitor your blood pressure at home if your provider recommends it
- ▶ Stay hydrated, limit sodium, and follow pregnancy nutrition guidelines
- ▶ Call **(281) 557-0300** with any questions or concerns